CLINICAL · EAR

# Myringitis

Myringitis is inflammation of the drum itself. In the bullous form, blood-filled blisters on the drum cause sudden, intense pain that settles in a few days. In the granular form, the drum's skin is replaced by wet granulation tissue that discharges without pain, and the middle ear behind it is healthy.

**AT A GLANCE**

- Bullous: severe pain with blisters on the drum.
- Granular: painless foul discharge with granulation.
- Rule out middle ear disease.
- Treat with analgesia and drops, or vinegar and removal.

## 1 CHAPTER ONE Overview

Disease of the drum, not the middle ear.

### 1.1 Definition

Inflammation confined to the tympanic membrane, sometimes with the adjacent deep canal skin, without primary disease of the middle ear or mastoid.

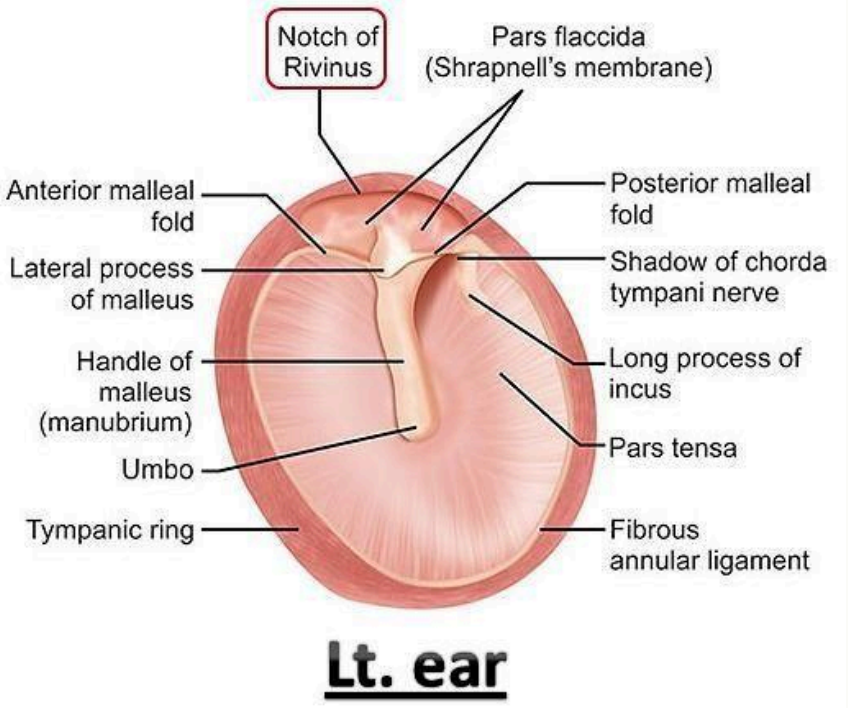



**FIGURE 1** Parts of the tympanic membrane.
Anatomy atlas · ENT Steps p. 36

## 1.2 Forms

### 1.2.1 Acute bullous myringitis

Blebs or bullae on the outer surface of the drum and the deep canal. Dhingra describes the same picture as *otitis externa haemorrhagica*.

### 1.2.2 Chronic granular myringitis

The epithelium of the drum is replaced by granulation tissue, without a perforation.

# 2 CHAPTER TWO Bullous myringitis

Blisters and pain.

## 2.1 Cause

It follows an upper respiratory infection. The organisms are those of acute otitis media: *Streptococcus pneumoniae*, *Haemophilus influenzae* and *Moraxella*. *Mycoplasma*

is the classic association, but it has not been proven.

WHERE THE BOOKS DIFFER · MYCOPLASMA

| Dhingra | Lee |
| --- | --- |
| Lists it as a possible cause. | The link is unproven. |

### 2.2 Clinical picture

Sudden, intense ear pain for three or four days that then settles, sometimes with a little bloody discharge when the blebs burst. Haemorrhagic bullae sit on the drum and deep canal. A third to two-fifths have a middle ear effusion as well.

### 2.3 Hearing

A sensorineural or mixed loss, often in the high frequencies, is common and usually recovers. Always check hearing.

### 2.4 Treatment

Analgesia; lancing the blebs relieves pain; keep the ear dry; topical drops to prevent secondary infection; treat acute otitis media if present.

## 3 CHAPTER THREE Granular myringitis

A wet, painless drum.

### 3.1 Cause

Usually *Staphylococcus*, *Pseudomonas* or *Proteus*.

WHERE THE BOOKS DIFFER · PREDISPOSING FACTORS

| Dhingra | Lee |
| --- | --- |
| Links it to wax, foreign bodies and external ear infection. | No predisposing factor has been identified. |

## 3.2 Clinical picture

Itching, fullness and a foul, painless discharge. Granulation tissue covers part of the drum, often posterosuperiorly; the drum may perforate and heal repeatedly. Hearing is normal or slightly reduced.

## 3.3 Diagnosis

Clinical, under the microscope. CT only if chronic otitis media or cholesteatoma has to be excluded.

## 3.4 Treatment

Regular cleaning, dilute vinegar irrigation and topical drops; cauterising the granulation with silver nitrate or trichloroacetic acid, or laser; removal of the granulation. Severe or recurrent cases need tympanoplasty with canalplasty. Recurrence is common after surgery.

## 3.5 Complication

Late fibrosis can blunt the anterior angle between the drum and canal wall, causing a conductive loss.

# 4 CHAPTER FOUR Differential diagnosis

Other painful or wet drums.

- **Acute otitis media:** bulging drum with poor mobility.
- **Chronic otitis media:** a perforation and middle ear disease.

- **Herpes zoster oticus:** vesicles on the concha and canal, facial palsy.
- **Otitis externa:** canal swelling and tragal tenderness.

# 5 CHAPTER FIVE For the examination

What the examiner wants to hear.

## 5.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- **Bullous versus granular,** and the symptoms of each.
- **Sensorineural loss** in bullous myringitis.
- **Treatment** of each form.

## 5.2 Common pitfalls

COMMON PITFALLS

- Not checking hearing in bullous myringitis.
- Calling granular myringitis chronic otitis media without examining under the microscope.

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